

Invented payer policy excerpts

Synthetic — not real data — AbarVa demo

Fictional document for Invented Payer Quartz (PY-01). Payer submission share in the synthetic baseline: 40%. These administrative examples are not actual payer policies or legal or clinical guidance.

POL-01 Eligibility D-EL

The invented coverage check must retain the subscriber-match response and coverage version effective on the service date. A later response does not retrospectively prove the earlier coverage check. An absent version routes the claim to patient access for review.

Proposed owner role: Patient access lead. Taxonomy preventability hypothesis: 80%, invented for discovery and awaiting human validation. This percentage is not a published payer performance statistic.

Review test: reconstruct the service-date decision from source versions, keep exceptions unresolved if required fields are missing, and record the reviewer before treating the rule as covered.

POL-02 Authorization D-AU

The invented authorization rule requires the reference identifier, validity interval and approved administrative service scope to match the submitted claim. An identifier alone is insufficient. A mismatch routes to the authorization team; no automated waiver is allowed.

Proposed owner role: Authorization team lead. Taxonomy preventability hypothesis: 75%, invented for discovery and awaiting human validation. This percentage is not a published payer performance statistic.

Review test: reconstruct the service-date decision from source versions, keep exceptions unresolved if required fields are missing, and record the reviewer before treating the rule as covered.

The other invented payer cohorts follow the same reviewed taxonomy for this fixture only. No real payer correspondence, payment incentive, tier or clinical programme is represented. Source links: 03-denial-reason-taxonomy.xlsx and 04-source-system-inventory.xlsx.

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Fictional document for Invented Payer Cobalt (PY-02). Payer submission share in the synthetic baseline: 30%. These administrative examples are not actual payer policies or legal or clinical guidance.

POL-03 Coding D-CO

The invented administrative edit compares the submitted procedure and billing modifiers to a reviewed local mapping. Overrides need a recorded reviewer and a reason. An edit does not recommend clinical treatment or replace professional coding review.

Proposed owner role: Coding manager. Taxonomy preventability hypothesis: 60%, invented for discovery and awaiting human validation. This percentage is not a published payer performance statistic.

Review test: reconstruct the service-date decision from source versions, keep exceptions unresolved if required fields are missing, and record the reviewer before treating the rule as covered.

POL-04 Documentation D-DO

The invented attachment rule requires a document reference and a completeness check before administrative submission. A missing reference routes to documentation operations. The fixture contains no actual clinical document and requires no real patient information.

Proposed owner role: Documentation operations lead. Taxonomy preventability hypothesis: 50%, invented for discovery and awaiting human validation. This percentage is not a published payer performance statistic.

Review test: reconstruct the service-date decision from source versions, keep exceptions unresolved if required fields are missing, and record the reviewer before treating the rule as covered.

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